

Prof. Dr. Qamruzzaman Chowdhury

MBBS, DMRT, FCPS

Senior Consultant, Oncology

Bangladesh Specialized Hospital.

BMDC Reg: A14404



HN ID : H12006095652

Name : SELINA KHAN

Age : 62Y 11M 20D

Gender : Female



Visit ID : C12406250578

Visit Date : 13 Jun 2024

Visit Time : 10:13 AM

Diagnosis

- Brain Metastases (Right posterior temporal region)- Post Operative (July 20). Lung primary with Nodal mets > Started Tab Osimertinib Since December/20 > Bone metastases (04/03/23)- Post Palliative RT> Osimertinib continued till OCT/23 > Suspected skin recurrence (Dec/23) > Restarted Osimertinib but progressive skin lesion> MET amplification> On Pal ChT

Clinical History

Histopathology: On Evercare, Brain tissue, right temporal lobe- Metastatic carcinoma, poorly differentiated, LVSI- Identified

On 25.12.23, BSHL, Tissue from rt side of neck & Right supraclavicular LN: Metastatic poorly differentiated carcinoma

Immunohistochemistry: On 11.08.20, Evercare; CK, CK20, CD10, TTF1, Arginase, EMA, Pancytokeratin, Vimentin, Ki67- **POSITIVE**. GATA3, CDX2, PAX8, WT1, GFAP, SYNAPTOPHYSIN, ER, PR, BCL2, S100-

NEGATIVE. A diffuse CK7/CK20+ profile favours a pulmonary or metastatic pancreatic, gastrointestinal, ovarian, bladder or midline carcinoma origin

Gene Study: On 28.11.20, Praava, Negative for- ML4-ALK, EGFR T790M, ROS1 gene fusion; **Positive for- EGFR Mutation**.

On 05.03.24, Reliance, Pathogenic mutation detected in **EGFR (Exon 19 del)** and **MET** gen amplification

Clinical stage: IV

Surgery: On 20.07.20, Fronto-temporo-parietal craniotomy done. Resection of the right temporal SOL with right temporal partial lobectomy done

Chemotherapy: Tab Osimertinib 80mg, Since Dec/20 to March 2024> PD > Zoledronic Acid started from 18.03.23 at monthly interval and **cycle 13 due on 25.05.24**.

Chemotherapy with Paclitaxel-Carboplatin x 3 cycle starting from 28.03.2024 and completed **C3 on 26.05.2024**.

Radiotherapy: Palliative RT 30Gy in 10fr in each site (right shoulder+ left femur) from 13.06.23 up to 26.06.2023 @LCH.

Vital

Height : 148 cm, Weight : 40 kg, BMI : 18.26 , BSA : 1.28 m2, BP : 100/60 mmHg

Investigation Notes:

On 11.08.20 PET CT: No hypermetabolic lesion in the operated right posterior temporal parietal lobectomy site of brain. Hypermetabolic centrimetric sized pre tracheal (SUV- 12.44), prevascular (SUV- 4.69), carinal (SUV- 14.59) and right hilar (SUV- 3.10) LNs are noted- suggest nodal malignancy. Hypermetabolic centrimetric sized soft tissue lesion in the antero-apical segment of upper lobe of right lung (18.17 x 25.9 mm, SUV - 9.34).

On 05.02.21 USG WA-NAD

Electronic Signature

Prof. Dr. Qamruzzaman Chowdhury

Chamber: 11:00AM - 6:00PM (Daily except Friday and Public Holiday), Report Viewing: After 1st visit within 7 days single visit

Free , Every report showing and follow up visit needs appointment. Please contact 10633

Printing Date & Time : 15-08- 2026 05:06 PM

Software By :: Mysoft Limited

Prof. Dr. Qamruzzaman Chowdhury

MBBS, DMRT, FCPS

Senior Consultant, Oncology

Bangladesh Specialized Hospital.

BMDC Reg: A14404



HN ID : H12006095652

Name : SELINA KHAN

Age : 62Y 11M 20D

Gender : Female



Visit ID : C12406250578

Visit Date : 13 Jun 2024

Visit Time : 10:13 AM

On 23.09.21, USG WA- NAD

On 23.09.21, CT Chest + Liver- Partial collapse in different areas of both the lungs with thickened pleura on both sides. No definite mass in any of the lungs. One enlarged lymph node in subcarinal region. As compared with previous PET CT- Reduction of size and number of lymph node.

On 29.03.22, PET Scan- No local residue on the area of craniotomy. Collapse consolidation of lingular segment of left lung . Hypermetabolic carinal and non avid B/L axillary LN suggest reactive nodes. both lobes of thyroid glands are enlarged with multiple non avid nodules within it.

On 23.02.23, MRI brain-Large post op CSF cavity surrounding by ischemic and gliotic change in rt temporo parietal lobe. No radiological evidence of recurrence. Evidence of rt temporo frontal parietal craniotomy. hypertrophied both inf. turbinate.

USG WA- Normal CXR- suggestive old pulmonary lesion (bilateral)

On 28.02.23, Bone scan: Suggestive of skeletal metastases at Rt shoulder & Lt acetabulum.

On 05.04.23 CXR- NAD

On 31.05.23, PET CT: No metabolic evidence of local recurrence in brain. Skeletal mets in right shoulder (SUV max 6.4) & left acetabulum (SUV max 9.2). A 3.1x1.7cm Ametabolic cavitary mass lesion in left lower lung. A subcentimetric hypermetabolic (SUV max 4.4) right axillary LN.

On 26.10.23, Whole body FDG PET CT: No definite recurrent mass or abnormal FDG uptake in either lung; Resolution of ametabolic cavitary lesion in left lower lung; Areas of scarring along with bronchiectic changes in both lungs (more marked in right upper & middle lobe along the mediastinal surface); Persistent few prominent right axillary lymph nodes showing almost stable FDG uptake (SUV max: 4.4 vs 4.8); Persistent sclerotic lesion in right scapula with metabolic regression (SUVmax: 2.18 vs 6.4); Remarkable metabolic regression (SUV max: 1.8 vs 9.2) of the lytic lesion in left acetabulum; No definite residual SOL or abnormal FDG uptake at the surgical site in brain, only non avid gliotic change and CSF filled cavity are there; No other suspicious new lesion or appreciable abnormal FDG uptake elsewhere in the body surveyed.

On 19.03.24, PET CT- Evidence of right fronto-temporo-parietal craniotomy having post operated photopenic area without any hypermetabolic lesion in fronto-temporo-parietal region suggest no local residue/recurrence. Intense hypermetabolic thickened cutaneous & subcutaneous lesion from the lower end of right side of neck up to back of neck & chest including skin over shoulder region suggest malignancy likely to be metastasis. No hypermetabolic lesion in post-treated either side of lung rather than post-treated fibrotic lesion suggest no local residue/recurrence. Multiple hypermetabolic right axillary and newly appeared left axillary & left sub-pectoral group of lymphnodes suggest metastases. Newly appearing hypermetabolic bony lesions in C7 & D2 vertebrae suggest metastases. No other visceral, nodal or osseous hypermetabolic lesion is noted anywhere else in the body.

On 12.06.2024, CBC- WNL, WBC- 3300, ANC - 1749

Finalization

Present Plan :

Electronic Signature

Prof. Dr. Qamruzzaman Chowdhury

Chamber: 11:00AM - 6:00PM (Daily except Friday and Public Holiday), Report Viewing: After 1st visit within 7 days single visit

Free , Every report showing and follow up visit needs appointment. Please contact 10633

Printing Date & Time : 15-08- 2026 05:06 PM

Software By :: Mysoft Limited

Prof. Dr. Qamruzzaman Chowdhury

MBBS, DMRT, FCPS

Senior Consultant, Oncology

Bangladesh Specialized Hospital.

BMDC Reg: A14404



HN ID : H12006095652

Name : SELINA KHAN

Age : 62Y 11M 20D

Gender : Female



Visit ID : C12406250578

Visit Date : 13 Jun 2024

Visit Time : 10:13 AM

On 26.05.24: Plan to switch to Capmatinib 200 mg- 1+0+1, as disease progression

Advice

- Follow up after EID vacation with patient & reports. Please take appointment before visit.
- Contact with Mr Ashik (Everest) 01714556810, 01909105740- for Capmatinib

R_x

- Tab Fexo 120 mg, 0+0+1, 15 days
- Tab Inflam 200 mg, 1+1+1, after meal, If pain
- Tab Esonix 20 mg, 1+0+1, before meal, 15 days
- Tab Gabapen 300 mg, 1+0+1, 15 days
- Tab Capmatinib 400 mg, 1+0+1, Continue

Investigation Advised

- CBC (URGENT)
- CT Scan of Neck- Chest including liver (with Contrast- If Creatinine OK) (Prof. Brig. Gen Dr. S M A Al-Muid/
Dr. Mariyam Sultana)
- USG of Whole abdomen

Electronic Signature

Prof. Dr. Qamruzzaman Chowdhury

Chamber: 11:00AM - 6:00PM (Daily except Friday and Public Holiday), Report Viewing: After 1st visit within 7 days single visit

Free , Every report showing and follow up visit needs appointment. Please contact 10633

Printing Date & Time : 15-08- 2026 05:06 PM

Software By :: Mysoft Limited